

COMPANY NAME | DATE

Findings & Recommendations for HPB - YPS

Content overview

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Our mission

This is your mission statement, your chance *to articulate your vision*. In a few words, identify what you do, why you do it, and who you do it for. *Be concise. Be confident.*

Add a few words expanding on the above. You can explain why this mission is important to you and your company and how you'll achieve it.

Why behaviour change needs to be tested, not just designed

The nature of behaviour change

Changing a habit is not like learning a fact. You can tell someone that sleep matters and they will agree with you. You can show them the research and they will nod. But knowing something and doing something are two very different things, and the gap between them is where most health interventions quietly fail.

This is especially true for youth. The youth in this study were not uninformed. They knew fast food was not great. They knew staying up until 2am on their phones was cutting into their sleep. Several of them could tell us, in their own words, exactly what a healthier routine would look like. What they could not always do was act on it, especially when they were tired, when their friends were doing something else, or when the easier option was just one tap away.

This is not a knowledge problem. It is a behaviour problem. And behaviour responds to very different things than information does.

The Stages of Change — where youth and parents actually sit

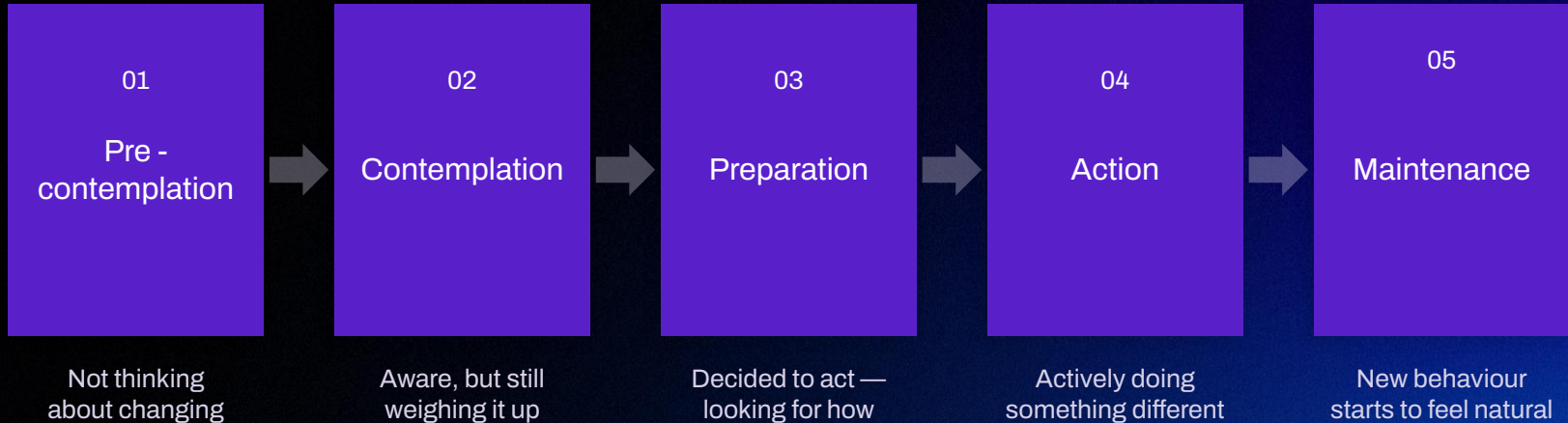
One of the most useful ways to understand why people do or do not change is through the Stages of Change model, developed by researchers Prochaska and DiClemente. It describes the psychological journey a person goes through before a new behaviour actually sticks, and it helps explain why the same intervention can work for one person and do nothing for another.

The five stages are simple. In the first stage, **Precontemplation**, the person is not thinking about changing at all. They may be aware of the issue, but they do not feel it applies to them, or the cost of changing feels too high to even consider. In the second stage, **Contemplation**, something has shifted. They can see that things could be different, but they are sitting on the fence. The third stage, **Preparation**, is when they have made a decision and are starting to look for a way to act on it. The fourth stage, **Action**, is when they are actively doing something different. And the fifth stage, **Maintenance**, is when the new behaviour has settled into routine and no longer requires conscious effort. This is what we mean when we say the healthy choice becomes the natural choice.

There is also a sixth reality that the model acknowledges, which is **Relapse**. Habits slip, especially during transitions like starting secondary school, moving up to a new year, or sitting exams. Relapse is not a failure of the person. It is a predictable part of the process, and it is why designs that build in recovery matter more than designs that assume a straight line.

Stages of Change

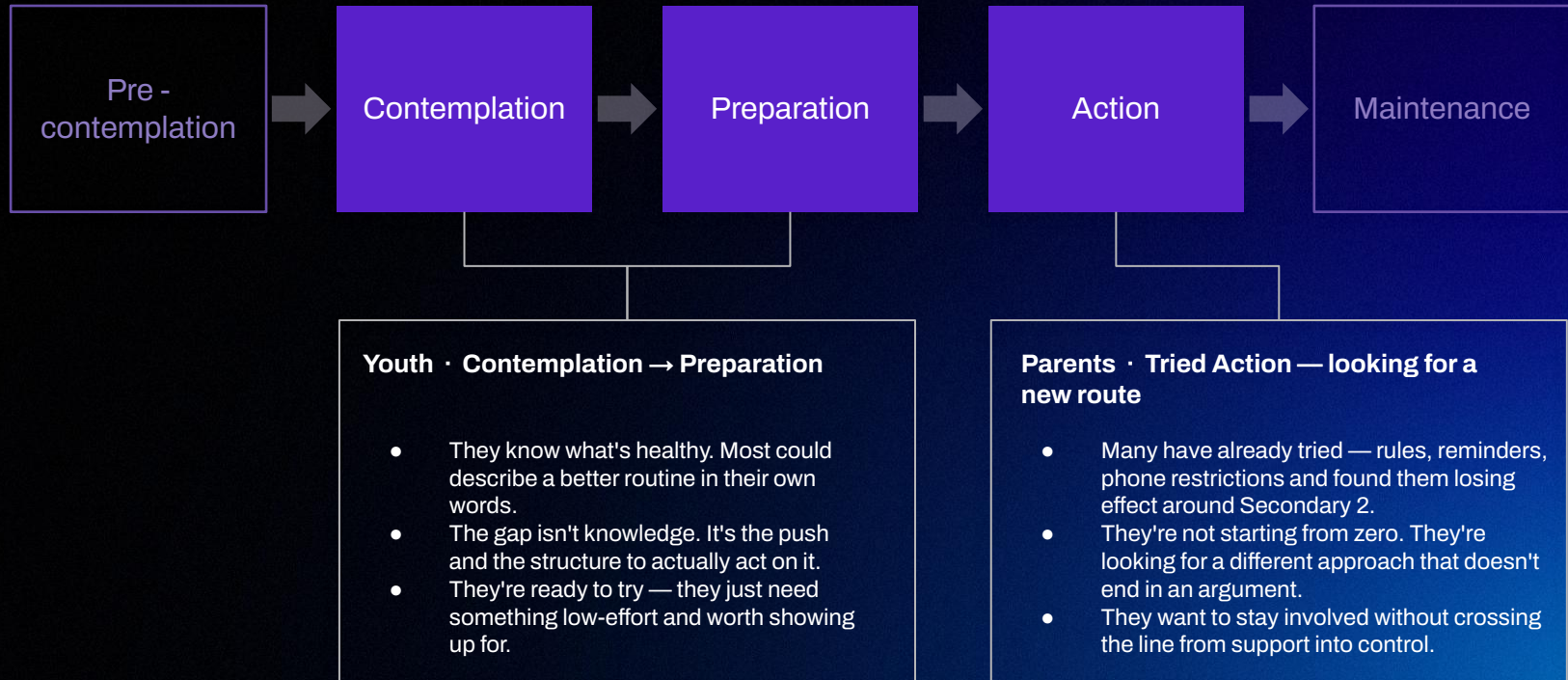
People don't change all at once. Where someone sits on this journey shapes what kind of support will actually land.



← Relapse is a normal part of the journey — habits slip, especially during transitions. The design goal is to give people something to return to.

Where are our youth and parents sit

People don't change all at once. Where someone sits on this journey shapes what kind of support will actually land.



What this mean on how we test

01

You can't design your way past this

Frameworks help us design well. But they can't tell us in advance where each person is on their behaviour change journey. That's what testing is for.

02

Real people respond differently at different stages

A tool built for someone ready to act will frustrate someone still sitting on the fence. The same concept can land completely differently depending on who is in front of it.

03

That's why we test before we scale

Testing surfaces what design cannot predict — a physical constraint, a trust issue, a timing barrier. These are things you can only know once a real person tries something in a real situation.

The knowing-doing gap

Most health interventions are built on one assumption: if people know better, they'll do better. The research says otherwise.

What youth **know**

- Fast food is not a good daily meal.
- Staying on their phone until 2am cuts into their sleep.
- Exercise helps with energy and mood.
- Sleep affects how well they function the next day.

What youth **do**

- Eat less healthily once school gets busy — cooking takes effort that isn't there.
- Scroll TikTok until midnight, because the urge to stop never comes.
- Skip exercise during exam weeks, because energy runs out first.
- Know they should sleep earlier. And don't.

The gap between knowing and doing is not a knowledge problem.
It is a behaviour problem, and it responds to very different design decisions.

Why willpower alone doesn't hold

For a behaviour to happen, three things need to be true at the same time.

Motivation

Does the person actually want to do this right now?

Not in general. Right now. A youth might want to sleep earlier in principle but at 11pm with TikTok open, the motivation to keep scrolling is stronger than the motivation to stop.

Ability

Is it actually easy enough to do in this moment?

Even motivated people don't act if the behaviour is too hard, too unfamiliar, or requires too many steps. If putting the phone down means finding something else to do with no clear alternative, most people won't.

Prompt

Is there something that triggers the behaviour at the right moment?

Even when motivation and ability are both present, behaviour often doesn't happen without a cue. A reminder. A friend asking. A physical object that signals it's time. Without a prompt, the moment passes.

Reality check: Motivation is not fixed. It shifts with time of day, mood, what friends are doing, and how tired someone already is.

Reality check: Difficulty is relative. What feels easy to an adult with a settled routine feels overwhelming to a 14-year-old managing school, friendships, and family all at once.

Reality check: Most healthy habits fail not because people don't want them, but because nothing in the environment prompts them when to start.

If any one of these three is missing, the behaviour will be difficult to shift, regardless of how much someone wants it to.

The same person responds differently in different situations

Context shapes behaviour more than character does. This is why solutions designed on paper often fail in the field.

Same youth, same day.

3pm, after school

Context: Just finished CCA. Friends are heading to the hawker. Still got energy and company.

M High. Friends are around. No reason to go home yet.

A Easy. Food is right there. No decisions to make.

P Friend says "let's go eat." The group moves together.

HEALTHY CHOICE HAPPENS

Goes to the hawker with friends. Eats a proper meal. Gets home in time to do homework.

10pm, in bedroom

Context: Homework done. Door closed. Phone on the pillow. Parents have stopped checking.

M Low. Tired. Wants comfort, not effort. The phone is already open.

A Hard. Putting the phone down means finding something else to do. There is nothing obvious.

P Nothing. No one is there. No cue to stop. The phone is the only prompt, and it says keep going.

UNHEALTHY DEFAULT WINS

Scrolls until 1am. Wakes up tired. Skips breakfast. Starts the next day already behind.

This is why context-sensitive testing with real people matters.
A solution that works at 3pm may do nothing at 11pm.

What the research pointed us toward

Across 24 conversations, the pattern was the same. Youth knew what healthy looked like. Most could describe it without being asked. The gap was rarely about information. It was the moment between knowing and actually doing something, and the environment around them wasn't reliably helping them cross it.

Parents hadn't stopped trying. Many had kept using tools that worked well when their children were younger: rules, reminders, set routines. By Secondary 2, those same approaches were starting to trigger resistance more than change for a number of our participants. The care was clearly there. What was harder to find was an approach that still worked.

What neither side could quite name was this: the handover happened, but the skills to fill it hadn't quite caught up yet. Independence arrived first. The ability to manage it followed later, sometimes not at all. That gap is where the drift lives.

The next section shows what we heard from the 24 people who helped us understand it.

"Once you take it back, I will go back to my sneakier behaviour."

- Amanda, youth co-creation

What we heard from 24 people (youth and parents)

Healthy habits don't slip randomly. They fade at three predictable moments

Parents don't step back all at once. Life changes in stages. And each stage removes a layer of structure before youth have learned to replace it.

P6 to Secondary 1

Structure steps back without warning

After-school care ends. Youth get a phone, a personal device, and more pocket money. Afternoons shift from adult-supervised to self-managed, before they have the skills to manage them.

"When I entered Sec 1, everything changed. New school, new friends, new timing. I had to figure everything out myself."

- Y03, 15yo male

Sec 2 to Sec 3

Peer influence fills the gap parents left

Parental visibility decreases. Sleep quality drops. Screens fill the gap. Meals get skipped because the easier option consistently wins. Without practical strategies, the pattern deepens.

"I used to swim three times a week. Then tuition started. Then exams. Now I don't do anything."

- Y09, 15yo female

Secondary 4 to JC / IHL

Those without their own routines are exposed

External structure has largely stepped back. Youth who built their own routines carry them forward. Those who didn't are still relying on someone else. By this point, that support is gone.

"My parents just... stopped checking. I didn't know if they trusted me or just gave up."

- Y12, 16yo female

We call this the drift: not a choice, but a gap that opens up when structure disappears before youth are ready to fill it themselves.

Youth know what healthy looks like. Three things keep getting in the way.

Across 12 youth and 12 parents, the same three barriers surfaced in different words, from different sides.

01

No one taught them how to manage it themselves

For most of our participants, someone was telling them when to eat, sleep, and move. Then secondary school started and that structure thinned out. The healthy habits that were held up by external support had no internal replacement. Without practice, they were the first to slip.

"I'm tired after school and gym. I just order food. Something I don't need to put work into. Almost every day."

- Y10, 15yo male

02

The unhealthy option is just easier

By Secondary 1, family meals and set bedtimes had mostly stopped for most of our participants. When the environment stops nudging you toward healthy, the easier choice tends to win: scrolling instead of sleeping, ordering instead of cooking.

"I experience a surge of energy at night, from about 7pm to 9pm. I find it difficult to fall asleep because I don't feel tired yet."

- Y01, 14yo female

03

Parents still care but their approach hasn't caught up

Parents haven't stopped trying. Many have kept using tools that worked well when their children were younger — rules, reminders, set routines. By Secondary 2, those same approaches were starting to trigger resistance more than change for a number of our participants. The care was clearly there. What was harder to find was an approach that still worked.

"I try to ask how school was, but she just says 'fine.' The conversations stopped somewhere around Sec 2."

- P09, parent of 15yo female

Co-creation confirmed the same gap, from both sides

6 youth and 6 parents co-designed with us. They arrived with different frustrations. They described the same problem.

What youth told us

- They already do the thing. Betting on weight loss with friends. Penalise each other for fast food. Keeping streaks. The accountability muscle is there. They just haven't aimed it at health.
- They need the how, not the why. Every idea they designed changed something around them the room, the routine, the friend group. Not one idea was "try harder."
- Peer pressure works both ways. The same force that pulls them toward screens can pull them toward healthy habits, if the social context is right.

What they agreed

- Screens affect sleep everyone can see it.
- Restriction without replacement makes things worse.
- Trust is what holds everything together. And it's fraying..
- Neither side knows how to start the conversation.

The handover happened. For many, the tools and skills to fill it hadn't quite caught up yet.

What parents told us

- They came in frustrated with themselves. Not just with their children. Many said the same thing: "I need to change myself too."
- Their tools are outdated. Rules built for a primary school child don't work on a fifteen-year-old. They know this. They just have nothing to replace them with.
- They want connection, not control. What they designed was shared rituals, not enforcement. A different approach, not a stricter one.

This is what we were designing toward

Our North Star

Healthy habits become the natural choice for youth — supported at key moments, shaped through shared routines, and sustained through growing independence

Three design principles to get there

Autonomy with guardrails

Design for shared structure, not control

Youth decide how things are done. Parents work alongside them. Neither side dictates. The structure is collaborative which means both sides are more likely to follow it.

Replace, don't restrict

Never remove without offering something better

Screens and fast food meet real needs: comfort, stimulation, belonging. Taking them away without offering an alternative just teaches workarounds. Design what fills the space.

Make it fit the day they already have

Strengthen existing behaviours before adding new ones

Youth and parents are already busy. If something doesn't slot into the after-school hangout, the bedtime window, or the dinner table, it won't get used. We design for real schedules, not ideal ones.

What the research pointed us toward

The findings from discovery didn't just describe a problem. They pointed toward the kind of solution that would and wouldn't work.

A solution that led with health messaging was unlikely to land. Youth weren't short of awareness about what was healthy. What many participants described as missing was a reason that felt relevant to them right now — something tied to energy, appearance, or how they showed up with their friends, not a long-term outcome they couldn't feel yet.

A solution that relied on parental enforcement was likely to produce more friction than change. By Secondary 2, approaches that felt like monitoring or control tended to produce resistance rather than cooperation. What parents in co-creation kept reaching for instead were shared rituals — things they could do alongside their child rather than impose on them.

A solution that required significant effort to start was unlikely to survive the after-school window or the bedroom at 10pm. For many of our participants, both felt like moments of low energy and high temptation. Whatever we designed had to be low-friction enough to work in those conditions, not in an ideal version of someone's day.

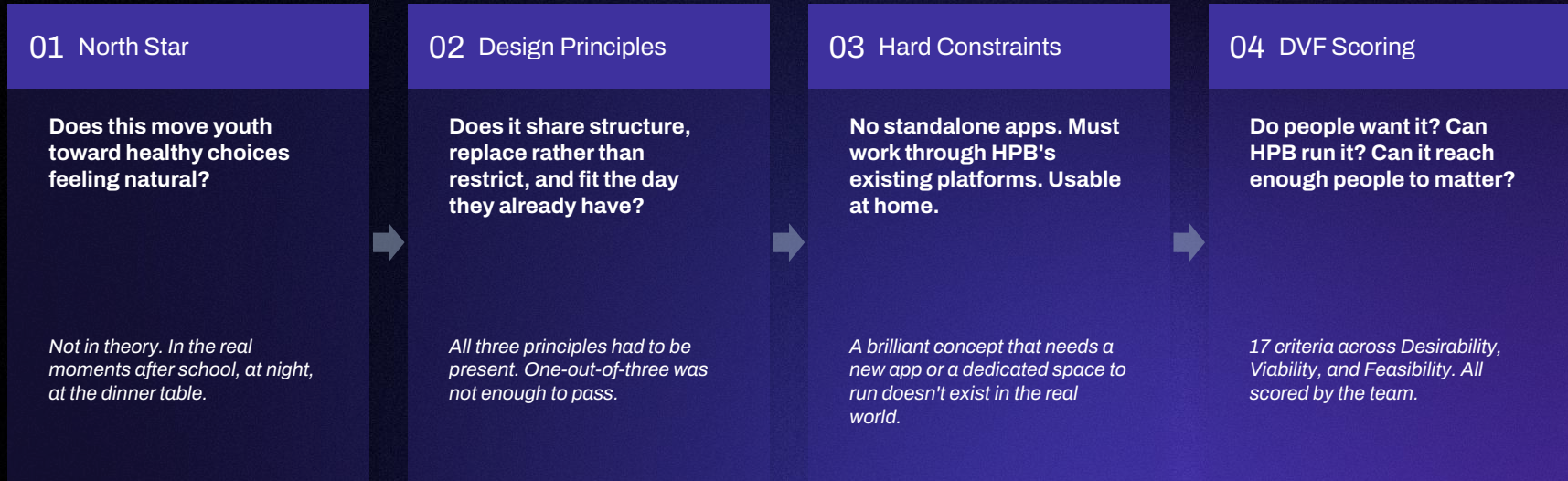
These weren't invented. They were grounded in what 24 people described across discovery and co-creation. The three concepts in the next section were filtered against these conditions specifically.

*"It only says why but not how. Never thinking about it."
Batch 2 youth, co-creation*

We tested physical concepts
through a screen and that matters

25 ideas passed through four filters. Three made it through.

Every idea from co-creation was put through the same test. Only concepts that passed all four moved forward.



25 concept directions → 4 filters -----> 3 concepts worth testing

Three concepts, each responding to something specific we heard

These weren't designed in a room. Each one answers a specific problem that came from real people.

C1

Bedtime Wind-Down

Build your bedtime routine. Together.

What it is

A physical magnet board where youth design their own wind-down routine. Youth picks the magnets, parent commits to their own column. 5-night framing. Weekly review built in.

Research finding it answers

Sleep is the most vulnerable habit. It has no driver once parental enforcement withdraws. Restriction backfires. When youth own the routine, they follow it.

Night window · 9pm onwards | Wells: Sleep, Screen, Bond

C2

Hook U Up

Pick a card. Find a buddy. Do it together.

What it is

A physical card deck for youth buddy pairs. 5 topic areas Glow Up, Bulk Up, Cut Season, Recharge, Feel Good. 14-day challenge, one card per day. No app needed.

Research finding it answers

Peer influence fills the 2–6pm supervision gap. Youth already challenge each other informally. Motivation is stronger when anchored to appearance, energy, and identity not health messaging.

After-school window · 2–6pm | Wells: Eat, Exercise, Screen

C3

We Open Up

A card game for parents and youth. No agenda. Just talk.

What it is

A family card game with three mechanics: Category cards start the conversation, Swap has both sides answer as each other, Spark goes deeper. Both sides answer the same questions no one is lectured.

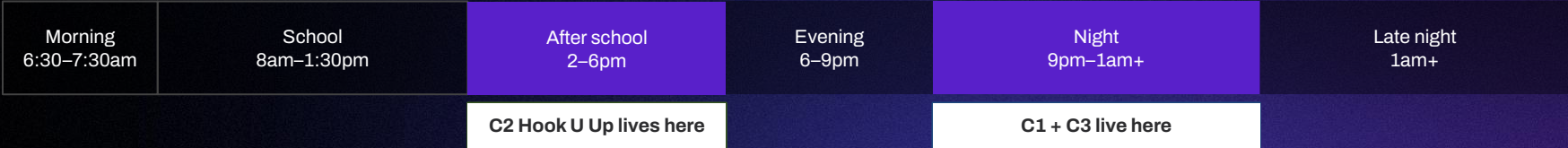
Research finding it answers

Parents want to help but their strategies were built for a younger child. Both sides want connection but neither knows how to start without it becoming a lecture or an argument.

Evening window · Dining table | Wells: Bond (unlocks Sleep, Screen, Eat)

Together they cover different windows, different relationships, and different wells. Each one fills a gap the others don't.

These three concepts were not chosen because any one of them does everything. They were chosen because together they cover the gaps.



Concept	Who	Window	Relationship	Wells	TTM Stage
C1 Bedtime Wind-Down	Youth + Parent	Night 9pm to 1am+	Shared both sides have a column, both sides commit	Sleep · Screen · Bond	Contemplation → Preparation
C2 Hook U Up	Youth + Peer	After school 2pm to 6pm	Peer-led no parent needed. Friends hold each other	Eat · Exercise · Screen	Preparation → Action
C3 We Open Up	Youth + Parent	Evening dining table, weekends	Shared simultaneous reveal means neither side dominates	Bond unlocks Sleep, Screen, Eat	Contemplation → Preparation

No two concepts target the same window or the same relationship. That's deliberate.

Before we get to the findings

Three concepts reached validation testing. Each one was grounded in the research, filtered through design principles, and built to work within the constraints HPB operates under. By the time they reached participants, they had already been through a lot of scrutiny.

What they hadn't been through yet was a real person trying them in a real session.

That matters more than it might sound. A concept can pass every design filter and still surprise you when it meets a specific family, a specific dynamic, or a specific Tuesday evening. Testing is not a final exam. It is the point where the work becomes honest — where assumptions get confirmed, challenged, or replaced by something more accurate.

The sessions ran from 9 to 13 March 2026. Twelve parents and nine youth. Individual sessions, each running about 60 minutes. The format was online, which shaped what we could and couldn't see — and that is covered in detail in the methodology section that follows.

What the next sections contain is not a verdict on whether the concepts work. It is an honest account of what happened when real people encountered them for the first time.

"This is something we have been looking for a long time."

TP12 Bhavani, parent validation session

How we tested, what we were
looking for, and what a screen can
and cannot show

We sat with 21 people. Each one came in with a different situation.

Every idea from co-creation was put through the same test. Only concepts that passed all four moved forward.

Twelve parents and nine youth. Individual online sessions, each running about 60 minutes. Sessions ran from 9 to 13 March 2026.

We recruited for range, not uniformity. Parents came from different family structures: single parents, dual-income households, families where the child was the eldest and families where a sibling had already gone through secondary school. Youth came from different school years, different CCA commitments, and different relationships with their own health habits. Some had routines they were proud of. Others knew exactly what they should be doing and weren't doing any of it.

That range was intentional. Behaviour change looks different depending on where someone is starting from. A concept that resonates with a parent who is actively searching for something will feel redundant to a parent who has already figured it out, and we needed to see both responses.

"This is something we have been looking for a long time."

- TP12 Bhavani, single parent. Daughter's sleep had been a source of real tension at home. She is exactly who this was designed for.

PARENTS · 12

- TP01 - Systematic family, co-manages son's bedtime routine together
- TP02 - Single parent, 14yo son, son has become more independently minded since secondary school
- TP03 - Parent of older children, finds connection harder now
- TP04 - Parent of daughter with ACL injury, exploring lower-impact alternatives
- TP05 - Structured family, daughter is largely self-managing her routine
- TP06 - Son has grown more distant since entering secondary school
- TP07 - Child under O-level pressure, parent cautious about adding new demands
- TP08 - Confirmed the 2026 school phone-in policy firsthand
- TP09 - Introverted child, conversations between them have narrowed
- TP10 - Daughter's routine already well established, sees little gap to fill
- TP11 - Self-aware about her own difficulty sustaining new habits over time
- TP12 - Single parent, sleep has been a real source of tension with daughter

YOUTH · 9

- TY01 - 14yo, highly motivated, connects health to identity and appearance
- TY02 - Curious, immediately imagined a wide range of use cases
- TY03 - Asked for a "sleepy" emotion option, most common feeling after school
- TY04 - 14yo, ACL injury, physical challenges need a skip or swap option
- TY06 - Drew the TikTok and Snapchat streak comparison herself without prompting
- TY08 - Under O-level pressure, tends to overthink challenges
- TY10 - Tech issue in session meant no direct hands-on interaction
- TY11 - Invented her own bet mechanic for C2 on the spot
- TY12 - CCA fatigue affects her evening routine on heavy training days

We tested physical concepts through a screen and that matters

We are not raising this to be cautious. We are raising it because it changes how the findings should be read.

All three concepts were designed to be held, placed, or handled. The magnet board is meant to be by the fridge. The card deck is meant to be shuffled and passed between friends. The conversation game is built around a simultaneous physical reveal where two people flip their cards at the same moment. None of that happened in these sessions.

Participants navigated prototypes on a laptop or iPad, clicking through screens instead of touching physical objects. For one participant, a technical issue meant the facilitator had to control the prototype entirely on their behalf yet they still completed ratings for all three concepts. These are not individual failures. They are systematic constraints that affected every session to some degree.

Three things we could not see through a screen:

The simultaneous reveal in We Open Up

No participant reached it in any session. The core mechanic of the concept both sides revealing at the same moment was never actually tested. What we measured was comprehension of the instructions, not the experience itself.

The physical drag-and-drop of the magnet board

Several participants could not engage with it at all. Those who did were clicking on a screen, which removes the tactile and spatial dimension of placing a magnet beside a bed at night.

The social moment of passing a card to a friend

C2 depends on a physical object moving between people. That dynamic simply cannot be replicated in an individual online session with a researcher present.

Four questions we were asking across every session

01

EAST

Is this easy enough, appealing enough, social enough, and timed right?

EAST breaks down the practical conditions for a behaviour to happen. Easy means low friction to start. Attractive means it connects to something the person already cares about. Social means it works with the people around them. Timely means it shows up at the right moment in the right context. A concept can pass three of four and still not work the weakest link is usually what breaks it.

02

B=MAP

Does this person have the motivation, the ability, and a prompt all at once?

Motivation, Ability, and Prompt all need to be present at the same moment for a behaviour to happen. We were looking at which of these each concept supplied and which it relied on the participant to bring themselves. A concept that assumes high motivation from everyone is fragile. One that creates its own prompt is much more robust.

03

Kano

Consider your brand and customer segments. Explain why you're ideally positioned to reach them.

Kano separates must-haves from nice-to-haves from unexpected strengths. Must-haves are things that cause rejection if absent participants don't mention them unless they're missing. Performance features improve satisfaction as they improve. Delighters are things nobody asked for but that produced the strongest positive reactions. TY11 inventing her own bet mechanic for C2 is a delighter. The QR proof failing is a must-have that broke.

04

Success Metrics

Did it do what we said it needed to do before we saw the results?

Before testing started, we set specific observable outcomes for each concept things a participant would need to demonstrate for us to say the concept was working. These were set in advance, not after. This kept the findings honest. Met means we saw it clearly. Partial means we saw something but not fully. Not Met means the evidence wasn't there which in an online session often tells us more about the format than about the concept.

How to read what follows

Before the findings, a note on what the ratings mean and what they don't.

Met

We saw clear evidence of this in the session. The participant demonstrated it, said it unprompted, or the observer independently noted it. More than one participant showing the same thing strengthens it.

Partial

There was evidence, but it was conditional or incomplete. The behaviour happened for some participants and not others, or it happened but with support from the facilitator. Partial is not a failure it tells us what conditions need to be in place for the concept to work.

Not Met

The evidence wasn't there in this format. This does not mean the concept cannot work. In several cases particularly for We Open Up Not Met reflects what online testing cannot show. A concept mechanic that was never experienced cannot be judged as failed.

A note on the 1 to 7 ratings

Participants rated each concept on a 7-point scale across five dimensions. These scores are directional, not conclusive. At least four participants gave numeric scores that did not match what they said out loud. One participant rated concepts she could not physically interact with due to a technical issue. Observer notes and verbal feedback carry more weight than the numbers.